

HealthConnect Clinic

Project Progress Report -Week 5

Project Management Track | Prepared by: Project Manager | Reporting Period: Week 5
Status: Planning stage delivered, pending Sponsor sign-off

1. Week 4 Recap

HealthConnect Clinic loses appointment capacity and patient engagement to missed appointments (no-shows), with no way to identify at-risk patients before they miss a visit. Week 4 delivered the Initiating process group -the Project Charter, Scope Statement, and Stakeholder Register -using the business scenario and central project question, plus a structural-level understanding of the Appointment Dataset and Clinic Knowledge Base.

Assumptions carried forward from Week 4: a common or derivable patient identifier exists; POPIA consent is in place; and clinic and internal IT/Data staff will have capacity to act on, and later sustain, the solution. Week 4 proposed that Week 5 move from Initiating into Planning -which is the focus of this report.

2. Week 5 Objective

The Week 4 Charter set scope, authority levels, and success criteria, but did not yet contain a schedule, a network diagram, a full risk/dependency register, or resource, communication, and quality plans. Week 5 was scoped to close that gap: build the WBS, a dependency-driven schedule, a network diagram with critical path analysis, a formal schedule risk analysis, the Resource, Communication, and Quality Management Plans, the cost baseline, and expand the Risk and Dependency Registers to full planning detail -closing with a Sponsor baseline sign-off gate.

3. What Was Delivered This Week

Every Week 4 activity was converted into a numbered, owned, prioritised task in the Task Tracker. Planning was formally added as its own phase (Phase 2) -it did not exist as a distinct phase before this week -and built out as the following 10 tasks:

Code	Task	Owner	Priority
2.1	Develop Work Breakdown Structure	Project Manager	High
2.2	Develop project schedule (durations & dependencies)	Project Manager	High
2.3	Build network diagram & critical path analysis	Project Manager	High
2.4	Conduct schedule risk analysis	Project Manager	High
2.5	Develop Resource Management Plan	Project Manager	Medium
2.6	Develop Communication Plan	Project Manager	Low
2.7	Develop Quality Management Plan	Project Manager	Medium
2.8	Develop cost baseline / Project Budget	Project Manager	Medium
2.9	Expand Risk & Dependency Register to full planning detail	Project Manager	High
2.10	Planning phase gate: baseline sign-off (scope, schedule, cost)	Project Manager / Sponsor	High

Alongside the WBS, schedule, network diagram, and the three management plans above, Week 5 also produced:

- A Project Budget with a cost baseline and S-curve, covering all 8 phases including Planning, and still within the Charter's R650,000 ceiling.
- A Resource Allocation Plan (allocation % and workload by phase and by week) and an Activity/Resource Gantt Chart.
- Updated Task Tracker, Timeline, Risk Register, and Dependency Register, reflecting real Week 5 status rather than Week 4's initial estimates.

Full detail -every one of the 42 activities across all 8 phases, with owner, priority, status, and % complete -is in *HealthConnect_Week5_Execution_Registers.xlsx* (Task Tracker tab).

4. Progress and Schedule Status

Phase 1 (Project Initiation) is Complete. Phase 2 (Planning) is Complete except the baseline sign-off gate (2.10), which is In Progress pending Sponsor approval. Phases 3–8 (Executing and Closing) are Not Started, scheduled to begin once 2.10 is signed off. The Updated Timeline (in the same workbook) carries a Status column reflecting this against every activity and milestone.

Key Finding: Revised Schedule Baseline

Running the schedule and network diagram properly (tasks 2.2–2.3) showed the project's true minimum duration is 118 working days -about 24 working weeks -finishing 2027-03-17, roughly 2 weeks longer than the originally assumed 20-week estimate. This is driven by Planning's own internal critical chain (WBS → Schedule → Network Diagram → Schedule Risk Analysis → Risk Register expansion → sign-off) and by Phase 1's Charter task genuinely needing both prerequisite reviews finished first. It is documented in full in the Schedule workbook's Read Me tab and logged as risk R13 (Section 5).

Overall Status

Area	Status	Commentary
Overall schedule	Amber	Planning delivered on time; the CPM finding (R13) means the downstream baseline needs re-confirming with the Sponsor before Phase 3 starts
Budget	Green	No external spend yet; the R650,000 ceiling and 10% contingency are unaffected -Planning was funded from within the existing PM overhead allocation
Scope	Green	No scope changes; Scope Statement holding
Risk	Amber	Three new risks this week (R11–R13); overall risk profile has increased slightly but every item has a named owner and mitigation
Stakeholder engagement	Green	Compliance Officer and IT Lead engaged as planned; Sponsor sign-off on the Planning baseline is the one open item

5. Risks and Dependencies

The Risk Register was expanded from Week 4's high-level list to full planning detail (scored, mitigated, owned). Three new risks were identified this week:

- **R11:** Early data-quality signals suggest more missing/inconsistent values in the appointment dataset than assumed at Charter stage.
- **R12:** The IT Lead has a capacity conflict with commitments outside the project, threatening both near-term (patient identifier) and later (deployment support) work.
- **R13:** The schedule risk analysis itself: the dependency-driven schedule needs ~24 weeks, not 20, which needs to be baselined rather than silently absorbed.

The Dependency Register gained one new entry (DEP-010: Sponsor sign-off on the Planning-stage baseline) and two status changes (DEP-001 and DEP-003 moved to Complete; DEP-002 moved to At Risk). A further dependency, DEP-009, was created this week to resolve the IT Lead capacity issue underlying R12 (see Section 6). Full detail is in the Updated Risk Register and Updated Dependency Register tabs.

Assumptions and Issues Review

Category	Item
Still relevant	Staff availability for the pilot (Phase 6) -unchanged; to be reconfirmed closer to Week 20. Internal team capacity post-handover -unchanged; to be confirmed ahead of Phase 8. All Charter-stage limitations

Category	Item
	(fictional/anonymised dataset, assistant limited to the approved Knowledge Base, no built-in allowance for a second pilot iteration) still apply.
Resolved	POPIA consent status -confirmed by the Compliance Officer (DEP-003 now Complete). Appointment dataset access -confirmed usable for analysis and modelling (DEP-001 now Complete).
Changed	The patient-identifier assumption -originally assumed straightforward, now understood to likely require a constructed fallback key rather than a clean existing match (DEP-002 moved to At Risk). The 20-week schedule assumption -replaced by the dependency-driven, CPM-verified ~24-week baseline (R13).
New this week	Data quality is more inconsistent than assumed (R1 escalated, R11 added). IT Lead capacity is more contested than assumed (R12, DEP-009). The true schedule is ~24 weeks, not 20 (R13, DEP-010).

Potential impact: if the identifier and data-quality issues are not contained, Phase 3 could overrun into Phase 4's window, which would tighten the already-thin float around Phase 5 (Model & Assistant Development). Both are logged with named owners and tracked at the next cross-track check-in. The schedule finding should be addressed by Sponsor sign-off on the 24-week baseline at the Planning phase gate (2.10) -not by quietly compressing the plan back to 20 weeks, which would consume float that Phase 5 doesn't have to spare.

6. Cross-Track Coordination and Collaboration

With four technical workstreams about to begin practical work in parallel from Phase 3 onward, coordination is run as follows:

- Each workstream provides a short weekly status input (progress, blockers, anything affecting another track or the schedule), which the Project Manager folds into the Task Tracker, Timeline, and Risk/Dependency Registers within 1 working day.
- The Resource Allocation Plan is used at each cross-track sync to check for overload weeks before they happen, rather than reacting after a resource is already double-booked.
- Anything touching the critical path, budget, or the 75% accuracy target is flagged to the Sponsor rather than resolved silently, per the Charter's change-control boundary.
- A bi-weekly cross-track sync (per the Communication Plan) resolves anything a written update alone can't -this week, that included the IT Lead capacity conflict (R12/DEP-009).

This week's specific collaboration was with the Data Analytics / Data Science track:

Question	Response
What was exchanged?	Early data quality findings -a higher-than-assumed rate of missing/inconsistent values, and uncertainty over a shared patient identifier across scheduling and reminder records.
Why was it relevant?	These findings directly affect two Project Management artefacts owned by the PM but dependent on technical input to stay accurate: the Risk Register (R1, data quality) and the Dependency Register (DEP-002, the patient identifier).
What changed as a result?	R1's likelihood was raised from Medium to High, a new risk (R11) was logged to track the effort impact on Phase 3, DEP-002 was moved to At Risk, and a new dependency (DEP-009) was created to resolve the underlying IT Lead capacity issue that the identifier work is blocked on.

7. Key Decisions Made

- Adopted the ~24-week CPM-derived duration as the recommended baseline rather than compressing the schedule to hold 20 weeks, because compression would consume float from Phase 5 (Model & Assistant Development), which already has the least buffer in the project since it spans the December recess.
- Logged the schedule finding itself as a risk (R13) and a dependency (DEP-010: Sponsor sign-off on the new baseline), rather than treating it as a private planning note -it has a real, named consequence for the Sponsor to weigh in on.

8. Changes to the Week 4 Approach

No changes were made to the Charter's scope or success criteria. Two structural corrections were made to the Week 4 artefacts:

- Planning was added as an explicit phase -it was previously missing entirely.
- All task dependencies were changed from bare WBS-style codes to a “code – name” format, so nothing requires cross-referencing another sheet to understand.

9. Major Challenges Encountered

- Realising that ‘Planning’ had been treated as an assumption rather than a scheduled body of work -it needed to be built out explicitly, with its own activities, owner, and duration, rather than folded silently into the Charter.
- Deciding how to present a schedule that is genuinely 2 weeks longer than what was previously communicated, without either hiding the finding or overreacting to it.

10. Remaining Work and Next Steps

Remaining This Week

- Obtain formal Sponsor sign-off on the Planning-stage baseline (scope, schedule, cost) -activity 2.10.
- Resolve the IT Lead capacity conflict (DEP-009) via escalation if not settled by the next check-in.
- Confirm the ~24-week schedule (finishing 2027-03-17) against the Charter's original delivery expectations with the Sponsor.

Proposed Focus for Week 6

- Close out the Planning phase gate (2.10) once Sponsor sign-off is received, formally starting Phase 3 (Analysis & Data Preparation).
- Monitor Phase 3 activities (data quality assessment, EDA, patient identifier resolution) against the baselined schedule, watching R1/R11/DEP-002 closely.
- Use the Resource Allocation Plan to check Phase 3/4 workstreams for overload before it happens, particularly around the Project Manager's own concurrent load.
- Prepare the Phase 4 (Solution Design) coordination checkpoints with the Data Science, ML Engineering, and Generative AI workstreams.